

BANGKOK HOSPITAL

Department of Cardiology — Consultation Report

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SECOND OPINION — CONSULTATION REPORT

Date of consultation: 1 March 2026 | Ref: BKH-CARD-2026-04291

PATIENT

Margaret Elaine CHEN, DOB 14/05/1967, British national. Currently admitted at Bumrungrad International Hospital (ref: BIH-IPD-2026-030847). Consultation requested by family (daughter Sophie Chen).

RECORDS REVIEWED

Bumrungrad admission notes (27 Feb), ECG tracings (27-28 Feb), echocardiogram report (28 Feb), laboratory results (27-28 Feb), Royal Brompton cardiology letter dated 12 January 2026, current medication chart. I did not examine the patient directly — this opinion is based on records review and a 15-minute video consultation with Dr. Siriporn Amara.

ASSESSMENT

I agree with the diagnosis of NSTEMI. The troponin kinetics and ECG findings are consistent. The echocardiographic deterioration (EF 62% → 45%) is concerning but not unexpected in the acute setting.

Having reviewed the imaging and laboratory data, I note that the patient is responding well to current medical management. Her haemodynamics are stable, pain has resolved, and there are no signs of heart failure progression.

REPATRIATION ASSESSMENT

Medical repatriation with a cardiac escort nurse and portable monitoring equipment to London Heathrow is **feasible within 48-72 hours** via dedicated air ambulance (estimated flight time BKK-LHR: approximately 11 hours 40 minutes with technical stop in Bahrain).

The Royal Brompton Hospital has superior interventional cardiology facilities and the patient would benefit from continuity of care with her existing cardiology team.

I would consider repatriation **advisable** given the availability of long-term cardiac rehabilitation in the UK, **provided the angiography on 3 March does not reveal multi-vessel disease requiring immediate surgical intervention** (CABG). If single or two-vessel disease is found and successfully stented, repatriation could proceed 48-72 hours post-procedure.

CAVEATS

- This opinion is contingent on angiography findings. Multi-vessel disease or left main stem involvement would change the recommendation.
- Repatriation must be by pressurised air ambulance with supplemental oxygen, continuous cardiac monitoring, and IV access maintained throughout.
- A receiving hospital (Royal Brompton or equivalent) must confirm acceptance prior to departure.
- I defer to Dr. Amara as the treating physician for final fitness-to-fly certification.

Dr. Kittisak Prasert, MD, FESC

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